Apollo HOSPITALS
TOUCHING LIVES

Discharge Summary

Dept. of GENERAL SURGERY

GENERAL INFORMATION

UHID	AHB.0000752138	**Patient Identifier**	AHBIP39054
Name	Mr. MOHSIN CHOWDHURY M	**Age**	49Yr 11Mth 26Days
Sex	Male	**Ward/Bed No**	LEVEL-6 , EXECUTIVE SUITE LEVEL 6,Bed no:6070
Address	d-7 abc bay view rd no 3 khulshi hills,Other-City-Bangladesh,Other-State-Bangladesh		

Dr. Saurabh Misra Dr Mukunda
MS
GENERAL SURGERY

Date of Admission 03-Jul-2017

Date of Discharge 07-Jul-2017

Diagnosis

MORBID OBESITY
HYPERTENSION
HYPERSOMNIA
DYSLIPIDEMIA

Surgery

Surgery / Procedure

Surgery / Procedure Name	Date
LAPAROSCOPIC GASTRIC BYPASS (LAP MGB)	04-Jul-2017

Type of Discharge Normal

Chief complaints morbid obesity

History of Present Illness Patient is a known case of morbid obesity, presenting now for surgery.

Past/Present/Family History k/c/o hypersomnia
k/c/o dyslipidemia

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Known case of DM	No
Known case of HTN	Yes
Known case of CAD	No
Known case of Asthma	No
Drug Allergy	Not Known

Clinical Examination

Pallor	No	**Icterus**	No
Cyanosis	No	**Clubbing**	No
Lymph nodes	No	**Pedal Oedema**	No
Respiratory Rate	18/min	**Temperature**	98.6°F
Pulse	60bpm	**B.P.**	130/90mm Hg
CVS	S1,S2(+),No murmurs.	**RS**	Bilateral air entry equal,NVBS(+), no added sounds.
Abdomen	Soft, nontender, no organomegaly, bowel sounds(+).	**CNS**	Conscious and oriented. No focal neurological deficits.

Treatment Given

Inj Perfalgan 1gm IV TID
Inj Pan 40mg IV OD
inj Supacef 750mg VI TID
Tab Nebicard 5mg OD
Tab Alpraz 0.5mg HS
Tab Olmat AM od
Neb Duolin TID
Neb Budecort 0.5mh BD

Course in the Hospital & Discussion

Mr. MOHSIN CHOWDHURY M, 49 year male, a known case of morbid obesity with above mentioned co-morbiditis, was admitted for surgical management. After clearance from Dr.Vikram Kamath (neurologist), Dr.Roshan Jain (Psychiatrist), Dr.Santosh(Endocrinologist),fitness from Dr.Kalpana (Physician) and consent, patient underwent Laparoscopic Mini Gastric Bypass under GA on 4/7/17. Patient withstood the procedure well. Post-op period was uneventful. Post-op gastrograffin study was normal and oral feeds were started on POD1. Patient is now being discharged in a hemodynamically stable condition with the following advice.

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OT Notes

PROCEDURE: Laparoscopic Mini Gastric Bypass under GA on 4/7/17.

INTRA-OP: - Liver enlarged.
- Small gastric pouch created.
- Gastrojejunal anastomosis done (155 cm).

Advice On Discharge

Diet	As per diet chart
Physical Activity	As advised
Discharge Advice	Tab. PANTODAC 40mg PO 1-0-1 x ~~15 days~~ (before food) Tab. UGESIC 1 tab Sublingual SOS (in case of pain) (max 12 hrly) Cap. ZEVIT 1 tab PO 0-1-0 x to continue Tab. NEBIVOLOL 5mg PO 1-0-0 to continue Tab. BIZORAN 20/5mg PO 0-0-1 to continue Tab. ARMODAFINIL 150mg PO 1/2-1/2-0 to continue Tab. RITALIN 10mg PO 1-1-0 to continue Tab. NIACIN+ECOSPRIN 500+75mg PO 0-1-0 to continue Tab. D 360 one a week for 8 weeks Tab. CCM 1-0-0 ~~after to 1 week stat after~~ 1 week Cap Andoxin 1-0-0 x 3 months
Follow up	Review with Dr.Saurabh Misra/Dr.Mukunda on Monday (10/7/17) in OPD with prior appointment. Review with Dr. Santhosh after 1 week with prior appointment.

CONDITION AT DISCHARGE

Vital Stable	Yes
Pain Score < 4	Yes
Surgical Wound	Clean
Ambulatory	Yes

In case of Emergency contact Emergency Physician on direct line or emergency reception at 1066.

Ask for Emergency Physician should you require urgent advice on a medical problem for e.g

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1. Fever > 101.5 F.
2. Onset of a new pain / worsening previous pain/chest pain / breathlessness.
3. Discharge from your operative wound (in case you had an operation).
4. Vomiting.

For any queries or appointment please call us at : 080-26304050

Dr. Saurabh Misra Dr Mukunda
MS
GENERAL SURGERY
Primary Consultant

PRESCRIPTION AUDIT * APPROVED Sign.

Please understand your discharge prescription from your doctor before using the medicines.
You can contact Emergency Room Physician, APOLLO HOSPITALS ENTERPRISE LTD at or .

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